

Obsessive Compulsive Disorder (OCD)

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Objectives

- Definition
- Epidemiology
- Aetiology
- Types of OCD
- Differential diagnosis
- Treatment

Definition of Obsessive Compulsive Disorder

- Obsessive-compulsive disorder is an anxiety disorder in which people have unwanted and repeated thoughts, feelings, ideas, sensations (obsessions), or behaviours that make them feel driven to do something (compulsions).
- Often the person carries out the behaviours to get rid of the obsessive thoughts, but this only provides temporary relief. Not performing the obsessive rituals can cause great anxiety.

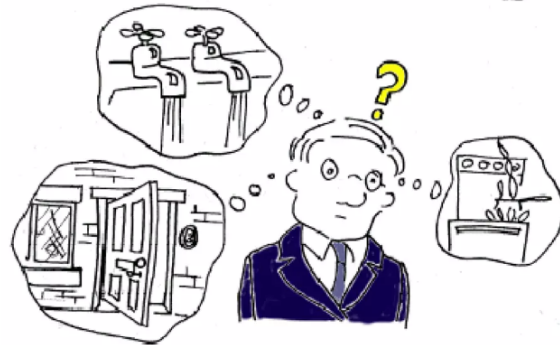
- **Obsession:** Recurrent, Persistent thoughts, impulses or images that the patient regards as absurd and alien, while recognizing them as the product of his own mind
- **Compulsions:** The motor component of an obsessional thought, also known as compulsive rituals

- **Obsessions increase anxiety ;where as carrying out compulsions reduce it; but when a person resists carrying out a compulsion, anxiety is increased.**
- People with disorder recognize that their actions are irrational or disproportionate
- **Egodystonic:** obsessions are described as egodystonic which means strange to the patient's thinking
- OCD is disabling but responsive to treatment
- Suicide is a risk for all patients with OCD

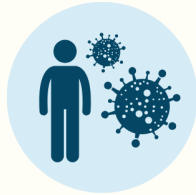
Obsessions

Affective Disorder

Contamination	45 %
Pathological doubt	42 %
Somatic	36 %
Aggressiveness	28 %
Sexual	26 %



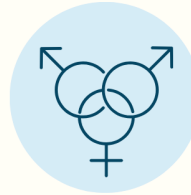
Common Examples of Obsessions



Contamination fears



Religious or Moral Obsessions:
Distressing thoughts related to these
areas, often causing guilt or fear



Unwanted sexual
thoughts or images



Persistent worries about
causing unintended harm



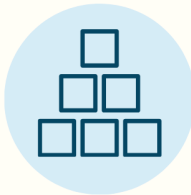
Intrusive images of violence,
sacrilege, or other distressing
scenarios



Intrusive images of
violence



Fear of Harming Others: Direct
and intrusive thoughts or images
about causing harm to others



Overwhelming need for
things to be "perfect" or
symmetrical



Repeated thoughts about
taboo or inappropriate
subjects

Compulsions

Checking
Washing
Counting
Symmetry & precision



Affective Disorder

63 %

50 %

36 %

28 %



Common Examples of Compulsions



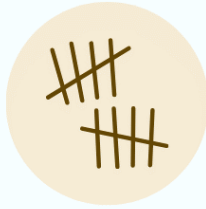
Hand Washing



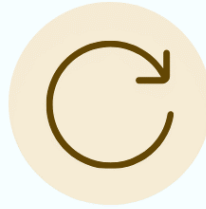
Prayer



Checking



Counting in specific
patterns



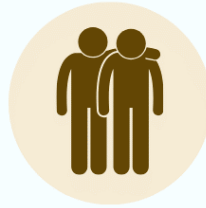
Silently repeating a
word



Cleaning



Body checking for
signs of illness

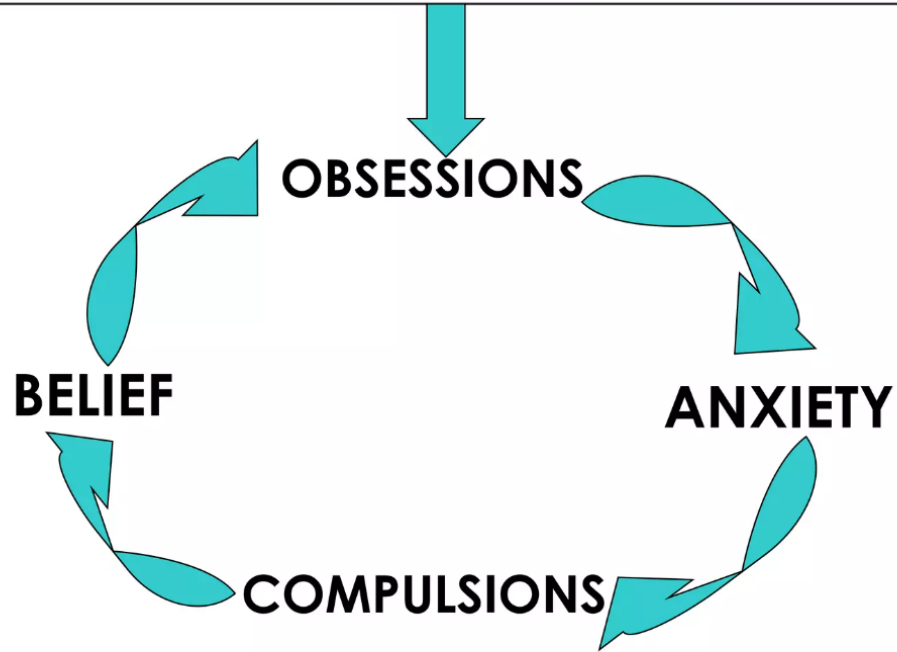


Reassurance
seeking



Confessing

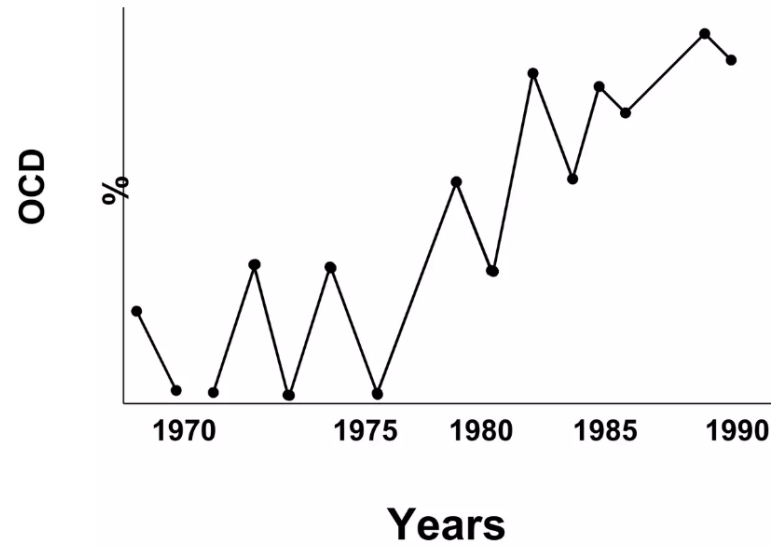
OCD Cycle



Epidemiology of OCD

- Prevalence rate is 2-3%
- OCD usually begins in adolescence or early adulthood, but it may occasionally begin in childhood
- In adolescents boys are more commonly affected than girls
- Mean age of onset is 20 years
- The onset is usually gradual and most patients have a chronic disease course with waxing and waning of symptoms in relation to life stressors
- Equally common among men and women
- 15% of patients have a chronic debilitating course with marked impairment in social and occupational functioning
- Up to 50% of patients with Tourette's disorder have coexisting OCD; however, only 5% of OCD patients have Tourette's disorder

Rate of Diagnosis of OCD

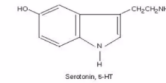


Aetiology of OCD

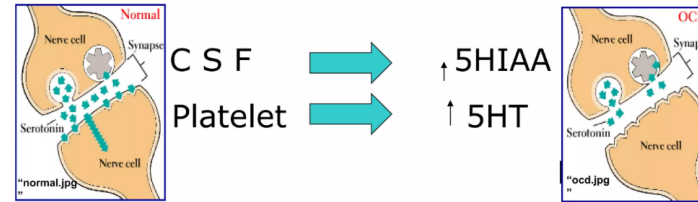
- **1. Biological factors:**
 - A- Neurotransmitters: serotonin
 - B- Brain imaging studies: increased activity in frontal lobes, basal ganglia(especially the caudate), and the cingulum

Neurobiological factors

A. Neurotransmitter Levels



Serotonin



- **2. Genetic:**

- A- Family studies: 5-7% of the parents of patients with OCD
- B- Twin studies: (monozygotic twin 50-80%, dizygotic twins 25%)

- **3. Organic factors:**

- Some evidence for organic brain disease associated with obsessive symptoms such as encephalitis lethargic

- **4. Premorbid personality:**

70% of patient with OCD have premorbid anankastic personality traits, cleanliness, rigid, checking personality

Types of Obsession

Presentation is heterogeneous, but there are specific patterns which are the major patterns

- **Obsessive Thoughts:** Repeated and intrusive words or phrases e.g.: Violent
- **Obsessive Rumination:** Repeated worrying themes of a more complex kind e.g.: about the world ending
- **Obsessive Impulses:** Repeated urges to carry out actions that are usually a dangerous, aggressive or socially embarrassing e.g. to shout obscenities in church, blaspheme in mosque
- **Obsessive Phobias:** Obsessive thoughts with a fearful content, e.g.: I must have cancer
- **Obsessive Doubts:** Repeated themes expressing uncertainty about previous actions, e.g.: gas tap, door...

Subtypes of OCD

**Contamination obsessions
with cleaning compulsions**



Hoarding



**Symmetry obsessions
with ordering compulsions**



**Obsessions
without visible
compulsions**



**Harm obsessions
with checking
compulsions**



Differential diagnosis of OCD

1. Medical conditions:

- Tourette disorder and other tic disorders
- Temporal lobe epilepsy(complex partial epilepsy)
- Head trauma
- Post-encephalitic complications

2. Psychiatric disorders:

- Schizophrenia
- Obsessive-Compulsive personality disorder
- Phobias
- Depressive disorders

Treatment of OCD

- **A. Pharmacotherapy is almost always indicated**
- TCA: Clomipramine (Anafranil)
- SSRIs: Sertraline, Paroxetine, Fluoxetine, Citalopram, Escitalopram and Fluvoxamine are effective
- Other drugs: Venlafaxine, MAOIs, Augmentation drugs
- **B. Standard antidepressant doses** of Clomipramine are usually effective, but higher doses of SSRIs are usually required, such as Fluoxetine 60-80 mg, paroxetine 40-60 mg, or sertraline 200 mg

- **C. Behavior therapy**, such as CBT cognitive-behavioral therapy, thought stopping, desensitization or flooding may also be effective. A combination of behavioral therapy and medication is most effective

It is rare for treatment to completely eliminate the symptoms of OCD, but significant clinical improvement in symptoms can occur, and the patient's functioning can be drastically enhanced

Prognosis:

- 2/3 improves by the end of one year
- Cases lasting more than 1 year usually run a fluctuating course, few months or several years
- Poor prognosis associated with (Anankastic personality traits, continuing stressful event in life, severe symptoms)

Other Illnesses Close to OCD

- **Obsessive compulsive personality disorder**
- **Generalized Anxiety disorder**
- **Anorexia Nervosa**
- **Hypochondriasis**
- **Pathologic skinpicking**
- **Trichotillomania**